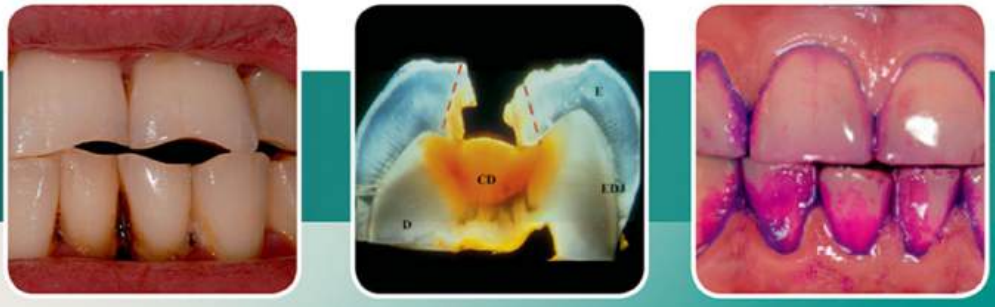
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Pickard's Guide to Minimally Invasive Operative Dentistry

Tenth edition



Avijit Banerjee
Timothy F Watson



Pickard's Guide to Minimally Invasive Operative Dentistry



Professor H. M. Pickard 1909–2002

Pickard's Guide to Minimally Invasive Operative Dentistry

TENTH EDITION

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Foreword

It was a 'great pleasure and honour' to have been asked to prepare the foreword for the previous edition of Pickard. To have been asked to prepare the foreword for the new-look, new-style tenth edition is a huge honour and pleasure, given that I have been selected to introduce this ground-breaking book in the rapidly developing field of patient-centred and oral healthcare team-delivered, biologically based, minimally invasive, minimum intervention (MI) care dentistry. To practise operative dentistry in any other way in the twenty-first century is not only old-fashioned, but contrary to the best interests of patients.

If Professor Pickard were alive today, I believe he would be delighted to be associated with the ways in which operative dentistry and relevant dental biomaterials science have evolved and developed since his pioneering days as a leader in the field, and to see the tenth edition of his book promote and encourage modern MI approaches to the clinical practice of operative dentistry. I also anticipate that Professors Bernard Smith and Edwina Kidd, who were entrusted with the 'Manual' by Pickard, will be most pleased to see this new edition pushing back the frontiers of the clinical practice of operative dentistry. As described in the accompanying preface by the present custodians of the Pickard legacy—Professors Avijit Banerjee and Timothy Watson—the 'Manual' has undergone major change during the preparation of the new, more user-friendly tenth edition, including a change in title to reflect all that is new in the state-of-the-art management of diseased and damaged teeth and tooth tissues. Professors Banerjee and Watson are to be congratulated not just on the excellence and attractiveness of the tenth edition of Pickard, but on their forward-looking approach and leadership in the promotion of patient-centred, minimally invasive, minimum intervention care dentistry.

In my foreword to the ninth edition of Pickard, I referred to a 'watershed between the traditional and modern art and science of operative dentistry.' With the publication of this tenth edition of Pickard, the transition from traditional, mechanistic, and traumatic to modern, truly tooth-preserving, conservative operative dentistry may be considered to have been largely completed, one of the major outstanding challenges being the translation of all that is described and beautifully illustrated in this book into the everyday clinical practice of dentistry. For established practitioners and teachers this may necessitate a fundamental change in thinking and approach. For students (present and future) a door has been opened to an innovative, much more challenging approach to patient-centred care. For future researchers, this book highlights the many different ways in which operative dentistry and relevant dental biomaterials science must continue to evolve.

From the style developed by Professors Banerjee and Watson, it is apparent that they fully appreciate the challenge that this tenth edition of Pickard poses in terms of putting aside traditional thinking and long-established procedures and techniques in order to embrace new, preventatively orientated concepts and principles. Of particular note is the introduction in this new edition of the '5Rs' concept to enhance and extend the life expectancy of restorations, and in turn teeth, through a slowing down of the so-called 'restorative death spiral.' If you are unfamiliar with the '5Rs' concept, that alone is sufficient justification for you to acquire and study this book.

I applaud the work of Banerjee and Watson and unreservedly recommend this new edition of Pickard to all members of the oral healthcare team. I very much hope that the new knowledge and understanding that it imparts will be widely and effectively translated by all team members into clinical practice in the best interests of existing and future generations of patients. For this to happen, it is hoped that teachers will revise their curricula accordingly, and funders of oral healthcare provision will critically review the extent to which their systems allow and encourage the practice of modern, evidence-based, minimally invasive operative dentistry as described in this book.

Professors Banerjee and Watson have produced a state-of-the-art text on operative dentistry. We must now rise to the challenge to practise twenty-first rather than late twentieth-century operative dentistry. Anyone who reads this book will be hard pressed to find good reason for continuing to practise 'old-style' traditional operative dentistry. The move to 'new-style' MI operative dentistry may be challenging, but it will bring countless benefits both to patients and to the profession.

Nairn Wilson CBE DSc (*h.c.*) FDS FFD FFGDP FCDSHK FACD FADM FHEA FKC

Preface to the tenth edition

The first edition of this textbook was published in 1962. From its origins, initially with Professor Pickard at the helm, the subsequent editions have always promoted operative dentistry principles that placed tooth preservation first and foremost. Since this time there have been major advances in the science underpinning our subject. These include a better understanding of the complex pathological processes that cause hard tissue disease, along with its detection, diagnosis, and the operative technologies and adhesive/sealing dental materials used to manage the damaged tissues. This *minimally invasive* biologically based approach is now recognized as the gold standard, and we have embraced this with the change of book title of this current edition, to *Pickard's Guide to Minimally Invasive Operative Dentistry*.

The ninth edition received a major update in style and content based upon the sound foundations from its previous editions authored by Professors Smith and Kidd. In this new edition we have responded to reader feedback and have made the following changes:

- We have included a chapter on the principles of the operative management of the badly broken down tooth, leading the reader towards an appreciation of the intimate link between direct and indirect restorations.
- We have expanded the final chapter covering the long-term clinical management/maintenance of direct restorations, using the minimally invasive '5Rs' concept—*review, refurbish, reseal, repair*, and *replace*.
- With respect to referencing the information in the book, we felt that traditional reference lists at the end of each chapter become rapidly outdated in areas that are undergoing continuous evolution. Therefore in this edition we have introduced QR code images to allow the reader to access with their mobile devices original and supporting material resources through digital media. The use of keywords for searching online databases will allow the reader to review references that will evolve dynamically.

This edition has been significantly enhanced by the inclusion of more high-quality images to help illustrate scientific concepts and clinical scenarios. We must thank Dr Louis Mackenzie, who has kindly provided many of these additions. In addition, we wish to thank our many colleagues who have allowed us to use their illustrations. They are acknowledged in the captions to the relevant figures, together with the source of the original publication where applicable.

In the previous edition we reinforced the link between prevention, operative dentistry, and overall patient care. This *minimum intervention* care philosophy continues to underpin the current edition, with increasing emphasis placed upon the differing important roles of the oral healthcare team. The operative skill set of a new dental graduate has evolved to encompass not only the techniques, materials, and science of minimally invasive dentistry, but also, increasingly, the behaviour management of their patients. Without patients taking responsibility for their oral health, even the best operative dentistry will fail, regardless of the materials used. In this regard, the amalgam debate has not gone away. Indeed, as a result of the United Nations Minamata Treaty in 2013, the global environmental impact of dental amalgam has led to recommendations for a phase down in its clinical use. The treaty has highlighted the need for increased dental research into caries prevention and alternative restorative materials in conjunction with better professional education in their use. We sincerely hope that this book goes some way towards achieving the latter.

A.B.
T.F.W.
October 2014

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